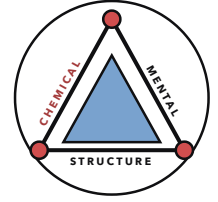
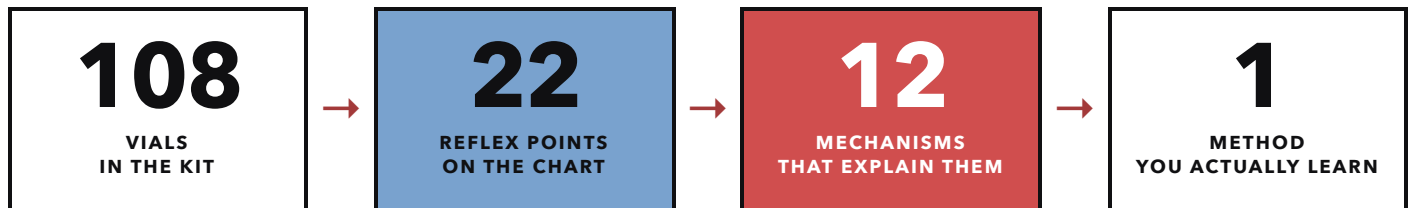


You do not have to memorize 108 things.



Open the kit and it looks like a wall. It is not. Everything in that box collapses down, and once you see the collapse you can put it to work this quarter instead of next year.



Every vial in that kit is an instance of one of **twelve stories**. Bile and fat. Acid and protein. Enzymes. Barrier. Fermentation. Stress rhythm. Thyroid. Detox conjugation. Mitochondria. Blood sugar. Membrane lipid. Immune signaling. That is the entire chemical leg of the Triad, and it fits on the back of this card.

WHAT YOU MEMORIZE, AND WHAT YOU LOOK UP FOREVER

MEMORIZE THESE THREE THINGS

- **The five steps** on the next page.
- **The twelve mechanisms** on the back.
- **The red flags.** This one is not optional. It is the only part where being wrong hurts somebody.

LOOK THESE UP, FOREVER, WITH NO SHAME

- Which vial.
- Which product.
- Which dose.

Practicing physicians look things up constantly. A doctor who checks a reference looks careful. A doctor who cannot explain the mechanism looks like a salesman.

THE ONE RULE THAT KEEPS ALL OF THIS DEFENSIBLE

The muscle test is the doorway. The biochemistry is the load-bearing wall. Never let a test result be the only thing holding up a recommendation. Confirm it with history, symptoms, exam, or a lab before you act. That is not us hedging, it is what ICAK's own 1992 Status Statement instructs: applied kinesiology "should enhance standard diagnosis, not replace it." Follow it and you are standing on the governing body of your own technique.

Five steps. That is the whole thing.

If you can answer all five out loud, you are ready to recommend. If you cannot, you are not, and that is useful information too.

0

Test in the clear.

Baseline first. The indicator muscle grades **facilitated** with no touch, no substance, no challenge. A muscle that will not test clear is unusable, and everything downstream of it is noise. Say *inhibited* and *facilitated*, never weak and strong. The muscle is not weak. The nervous system is not adapting it.

1

Find a second witness.

"What else in this patient points the same direction?"

This is the step everyone skips and it is the one that separates a doctor from a technician. History, symptom pattern, diet, medications, exam, or a lab. **One finding is a hypothesis. Two findings that agree is a clinical picture.**

2

Name the mechanism out loud.

"What is this system trying to do, and what is it short on?"

Not "the gallbladder is weak." Instead: "bile emulsifies fat into micelles, and without micelles you absorb neither your dietary fat nor the A, D, E and K riding in it." The twelve mechanisms on the back are the whole vocabulary you need.

3

Point at the ingredient that does that job.

"Which specific active touches the mechanism I just named?"

Read the panel. Do not trust the category name. Example worth knowing: Lipo-Flow sounds like it contains bile. It does not. It is choline, artichoke, dandelion and taurine, which help you *make and move* bile. Ox bile is in Digestive Complete. Different patients, different bottle.

4

Name the dose and the endpoint before they leave.

"What should change, and by when?"

"Take this and see how you feel" is not a plan. **Two to four weeks with one named marker** is a plan. Decide the marker in advance, then actually go back and check it.

AND THE QUESTION UNDERNEATH ALL FIVE, EVERY SINGLE TIME

Is there anything here that should not be treated with nutrition at all? Right upper quadrant pain with fever and jaundice is not a bile case. Unexplained weight loss is not an adrenal case. Chest pain is never a supplement conversation. Rectal bleeding, blood in stool, a new neurological deficit, a rigid abdomen: those are referrals, today, and nothing from a bottle. **Learn the red flags before you learn the products.**

SAY IT THIS WAY WHEN SOMEONE ASKS WHAT YOU ARE DOING

"The test told me where to look. What convinced me was that three other things you told me pointed the same direction. Here is why I think this helps, and here is what we should see change in about a month. If it does not change, that tells us something too."

The twelve mechanisms

Learn these and you can explain any finding in the kit. The right-hand column is the sentence you actually say to a patient. Say it in your own words once you own it.

#	SYSTEM	THE MECHANISM	THE SENTENCE YOU SAY	REACH FOR
1	Bile & fat	Bile acids emulsify fat into micelles. Micelles are <i>required</i> to absorb fat and vitamins A, D, E, K.	<i>"Every fat-soluble thing you swallow has to ride in on bile. If bile flow is poor, you can take the right vitamin forever and never absorb it."</i>	Lipo-Flow (make/move) · Digestive Complete (ox bile)
2	Gastric acid	Acid activates pepsin, frees B12 for intrinsic factor, solubilizes iron, zinc, calcium, magnesium, and blocks organisms.	<i>"Low acid does not just cause discomfort. It quietly costs you B12 and iron, because you need acid to unlock them from your food."</i>	HCl Support (betaine HCl + pepsin)
3	Pancreatic enzymes	Lipase, protease, amylase into the duodenum against a bicarbonate buffer. Insufficiency shows as fat in the stool.	<i>"If your stool floats or looks greasy, that is undigested fat. It means the enzymes are not keeping up with the meal."</i>	Digestive Complete · Lipo-Complex
4	Gut barrier	Enterocytes turn over every 3 to 5 days. Glutamine is their preferred fuel. Tight junctions control what crosses.	<i>"That lining completely rebuilds itself every few days. That is why it responds fast, and why it needs raw material to rebuild with."</i>	Dynamic GI Integrity (L-glutamine 3g)
5	Fermentation	Colonic bacteria ferment fiber into short-chain fatty acids. Butyrate is the colonocyte's own fuel.	<i>"Your gut bacteria turn fiber into the exact fuel your colon lining runs on. Low fiber starves that loop from both ends."</i>	Dynamic Fiber · Spore Probio
6	HPA axis	CRH to ACTH to cortisol, with a daily rhythm and feedback. Under load the pattern shifts, especially the morning rise.	<i>"This is a rhythm problem, not an organ running out of fuel. Your morning curve is flat and we can work on that."</i>	Stress Essentials Adrenal Renew · Cortisol Pro
7	Thyroid	Tyrosine on thyroglobulin is iodinated using iodine and peroxide. Selenium both protects the gland and activates T4 to T3.	<i>"Making thyroid hormone generates peroxide inside the gland, so the gland needs selenium to protect itself while it works."</i>	Thyroid Complex · Organo Iodine
8	Detox conjugation	Phase I exposes a reactive group. Phase II conjugates it to something water-soluble. Driving I without II raises the reactive load.	<i>"We are not just flushing something out. We are making sure the second step can keep up with the first, or you feel worse, not better."</i>	Dynamic Detox · LiverCare
9	Mitochondrial	CoQ10 carries electrons in the transport chain. It is made by the mevalonate pathway, the one statins inhibit.	<i>"Your statin works on the same pathway your body uses to make CoQ10. That is a known consequence, not a coincidence."</i>	CoQ10 200 mg
10	Insulin signaling	Berberine activates AMPK, the cell's low-energy sensor: more glucose uptake, less hepatic output. Chromium participates in signaling.	<i>"This is a signaling problem before it is a sugar problem, which is why your fasting glucose can look fine for years."</i>	Berberine Pro · Gluco IR
11	Membrane lipid	DHA is a structural phospholipid concentrated at the synapse. The brain is ~60% lipid by dry weight.	<i>"Omega-3 is not a supplement for your brain, it is a building material your brain is literally made of."</i>	Omega Pure EPA-DHA 720
12	Immune signaling	Vitamin D receptors sit on most immune cell lines. Zinc is required for lymphocyte development and function.	<i>"Vitamin D is not really a vitamin here, it acts like a signal your immune cells are waiting to receive."</i>	Immune Support · D3 5000 with K2

BEFORE ANY OF THE TWELVE, THERE ARE FIVE

Essential Multi · D3 with K2 · Omega-3 · Probiotic · Fruits & Greens. Fix the foundation before chasing anything specific. Three of those five are fat-soluble or fat-dependent, which means they need mechanism #1 working before they can even get in. That is why "I've been taking vitamin D for eight months and my level did not move" is a bile finding, not a dosing failure.

Run of show · Thursday August 6 · AK Club, Mod 7B, 11:00-1:00

THE ONE THING THAT SHAPES THIS ROOM

They come straight out of OSCE. DC Assessment Day runs Tuesday and Wednesday, August 4 and 5, so the entire student body has been examined for two days and walks in on Thursday with nothing left. **Keep it to twenty minutes and get the kits into their hands fast.** This is the worst possible week for a lecture and the best possible week to be the person who made something easy.

Kits are in hand. Both are on your NetSuite sales record. Grab them on the way out the door.

THE TWENTY MINUTES

1

Open the kit and say the collapse. Two minutes.

"There are 108 vials in here. You are not going to memorize 108 of anything, and I am not going to ask you to. They collapse into twelve." Then hand out the card and let them see it. **This is the whole wow. Do not bury it after twelve minutes of preamble.**

2

Run one mechanism all the way through. Six minutes.

Use gallbladder and bile. It is the most visual, it has a perfect objective marker, and it contains the Lipo-Flow surprise. Land the punchline: **Lipo-Flow has no ox bile in it, and it is still the right call for the patient who makes bile but moves it poorly.** That single fact teaches "read the panel" better than any lecture on reading panels.

3

Put it in their hands. Eight minutes.

Two kits, so two groups. Let them run the method on each other while you walk between them. Do not correct technique, they have an advisor for that. **Correct the explanation.** When someone finds something, ask "okay, why?" That question is your entire product.

4

Close with the ask, and capture names. Four minutes.

"This is edition one and you are the editors. Tell me what is wrong with it and what you actually want next." Then pass the sheet. **Names and emails are the entire point of the visit** — a kit you hand over with no capture is a donation, not a beachhead.

ANSWERS YOU NEED TO WALK OUT WITH

- **Is Lynda Davis the president?** She does not appear anywhere in your mailbox or on any public record. Engage lists **Tiffany Korb** as the contact. Just ask the room who holds which office and write it down.
- **Who is "Dr. Strong"?** That is the advisor name on Engage, with no first name and no faculty page. A 2022 article said Dr. Trent Brumbaugh. The advisor is your route onto the approved guest speaker list, so you need the real name.
- **How many active members?** Never published anywhere. Determines what a real sponsorship is worth.
- **Are they using Dr. Chris Montanaro's 100HR Foundations course?** His link is in their Instagram bio. If yes, you complement it and never compete with it.
- **Do they want the 12-week meeting curriculum?** If yes, that is the thing that scales past you.

WHAT YOU ARE ACTUALLY BUYING WITH TWO TEST KITS

Not goodwill from one club. Life University graduates roughly **300 chiropractors a year** and requires exactly **two nutrition courses, six credit hours**, all finished by quarter five. They do not enter the clinic until quarter nine. **That four-quarter gap is the product.** The AK Club is simply the room where the gap is most obvious and the students most want it closed. Close it here, prove the format, then take the same format to Functional Health and Functional Nutrition, both founded in 2024 and both running on guest speakers with no incumbent sponsor.